



Network Bulletin: August 2016

network bulletin

Important updates from UnitedHealthcare to health care professionals and facilities



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UnitedHealthcare respects the expertise of the physicians, health care professionals and their staff who participate in our network. Our goal is to support you and your patients in making the most informed decisions regarding the choice of quality and cost-effective care, and to support practice staff with a simple and predictable administrative experience. The Network Bulletin was developed to share important updates regarding UnitedHealthcare procedure and policy changes, as well as other useful administrative and clinical information.

Where information in this bulletin conflicts with applicable state and/or federal law, UnitedHealthcare follows such applicable federal and/or state law.

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Front & Center

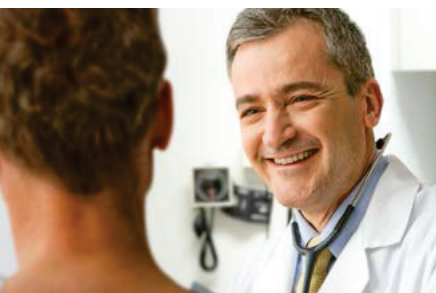
Communication Effectiveness Survey

We understand the volume of communications you receive from payers on a wide variety of topics and want to do whatever we can to streamline those you receive from UnitedHealthcare as well as improve those we do send. Please take a few moments to answer our brief three-question survey so we can continue to improve our communications to you.



The survey is available online at uhcresearch.az1.qualtrics.com/jfe/form/SV_a2TkHLsEbvfxGgB. Thank you!





Front & Center

Reminder: Please Complete CMS-required Model of Care Training

Each year UnitedHealthcare provides Special Needs Plan (SNP) Model of Care training required by the Centers for Medicare & Medicaid Services (CMS) for all our SNP network care providers. Please complete this year's training by Dec. 16, 2016.



Go to UnitedHealthcareOnline.com > Tools & Resources > Training & Education > Medicare & Medicaid Topics. Click **2016 Special Needs Plan Model of Care Training** to register or to read an FAQ about which care providers need to take the training.



If you have questions about whether you are required to take the Model of Care training or if you treat members in Special Needs Plans, please call 888-878-5499 or email us at snp_moc_providertraining@uhc.com.

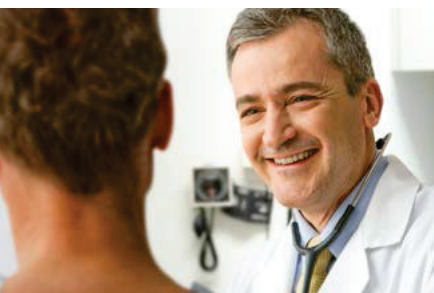
Reproductive Resource Services Guideline Revised

Effective May 5, 2016, the Reproductive Resource Services Medical Necessity Clinical Guideline: Infertility was revised based on current clinical evidence and expert panel input. Key revisions included:

- Additional indications for the use of Letrozole
- Additional indications for the use of gonadotropins
- eSET indications
- New section on Pre-Implantation Genetic Testing

This revised clinical guideline is available at UnitedHealthCareOnline.com > Tools & Resources > Policies, Protocols and Guides > Medical & Drug Policies and Coverage Determination Guidelines – Commercial > **Clinical Guidelines**.





Front & Center

New Link Applications Launching; Some UnitedHealthcareOnline.com Functions Retiring

Starting in September, we will introduce new applications on Link – your gateway to UnitedHealthcare's online tools and resources.

The new **eligibilityLink** and **claimsLink** applications will offer enhanced features not available on the current Link applications, as well as a redesigned user interface that is faster and easier to use.

Additionally, prior authorization and notification is now available as a separate **Prior Authorization and Notification** application. These functions will continue to be available as part of the Eligibility & Benefits application as well.

Once you've had a chance to try the new applications, we'll start to retire the corresponding functions on **UnitedHealthcareOnline.com**: Eligibility & Benefits, Claim Status and Claim Reconsideration.

EligibilityLink and claimsLink

The new eligibilityLink and claimsLink applications will help make it faster and easier to perform online transactions using Link. These new applications will not launch to all users at the same time; when they are available to you, we will email you with more details and information about webinar training sessions.

eligibilityLink has features you use today on the Eligibility & Benefits application, plus:

- Redesigned screens based on feedback from Link users
- View previous plans – up to 18 months in the past
- Switch to claimsLink without re-entering the member information

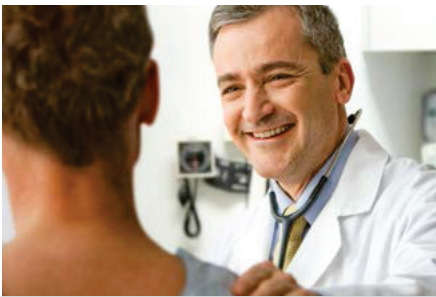
claimsLink combines the features of the Claims Management and Claims Reconsideration applications, plus:

- Redesigned screens based on feedback from Link users
- Switch to eligibilityLink without re-entering the member information
- Create a new claim reconsideration request or view an existing one from the claim detail screen
- View the care provider's network status for a member, when available
- Create your own claim view by selecting which information is displayed
- View additional claim letters

UnitedHealthcareOnline.com Functions Retiring

With the introduction of these new applications, we'll soon start retiring the Eligibility & Benefits, Claim Status and Claim Reconsideration functions on UnitedHealthcareOnline.com. If you use those functions on UnitedHealthcareOnline.com, you will see a pop-up box inviting you to try the new Link applications. After you have tried eligibilityLink or claimsLink twice, another pop-up box will invite you to make the new Link application your permanent view – which would make it the tool you use for all related transactions. You will have the opportunity to try the new applications for about a month before the functions are removed from UnitedHealthcareOnline.com.

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Front & Center

New Link
Applications
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The Eligibility & Benefits, Claims Management and Claims Reconsideration applications on Link will continue to be available into 2017, along with the new eligibilityLink and claimsLink applications. More new features are in progress, and we will continue to make improvements based on the feedback we receive from users.

Learn More

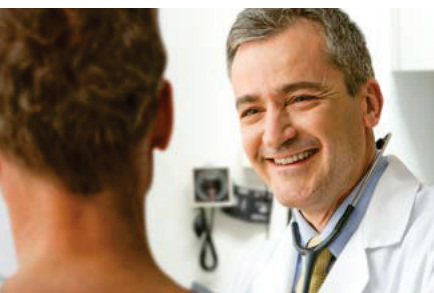


If you're new to Link, you can get more information about using it at **[UnitedHealthcareOnline.com > Quick Links > Link: Learn More.](#)**



If you have questions, please call the UnitedHealthcare Connectivity Help Desk at **866-842-3278**, option 3, 7 a.m. – 9 p.m. Central Time, Monday through Friday.





Front & Center

New Clinical Laboratory Improvement Amendments (CLIA) Identification Requirements Policy

In alignment with Centers for Medicare & Medicaid Services (CMS) and CLIA requirements effective Nov. 1, 2016 for participating providers and Aug. 1, 2016 for non-participating providers, UnitedHealthcare will implement a new reimbursement policy that will be applicable to all laboratory services submitted on either a CMS 1500 claim form or HIPAA 5010 837 P claim file.

The new reimbursement policy will apply to UnitedHealthcare Commercial and Medicare Advantage members.

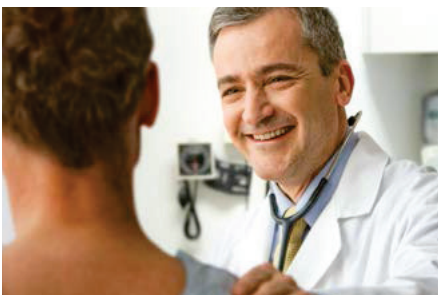
The policy requires that all claims for laboratory services include the Clinical Laboratory Improvement Amendments (CLIA) number for the servicing care provider. The laboratory servicing provider's physical address also will be required if the address differs from the billing provider's address noted on the claim. The billing or servicing provider address must match the address associated with the CLIA ID number.

Claims Submission Process

Providers should use the following guidelines to submit their CLIA identification number and servicing provider location information on claims for UnitedHealthcare members.

Claim Format and Elements	CLIA Number Location Options	Ordering Provider Name and NPI Number Location Options	Servicing Laboratory Physical Location
CMS-1500 (formerly HCFA 1500)	Must be represented in field 23	Submit the ordering provider name and NPI number in fields 17 and 17b, respectively.	▪ If the servicing address is not equal to the billing provider address: submit the servicing provider name, full physical address and NPI number in fields 32 and 32A, respectively. ▪ The servicing provider address must match the address associated with the CLIA ID entered in field 23.
HIPAA 5010 837 Professional	Must be represented in the 2300 loop, REF02 element	Submit the ordering provider name and NPI number in 2310A loop, NM1 segment.	▪ If the servicing address is not equal to the billing provider address: the physical address of the servicing provider must be represented in the 2310C loop. ▪ The servicing provider address must match the address associated with the CLIA ID submitted in the 2300 loop, REF02.
HIPAA 5010 837 Institutional	Not applicable for institutional claims		Not applicable for institutional claims

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Front & Center

New Clinical Laboratory Improvement Amendments (CLIA) Identification Requirements Policy

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CLIA regulatory requirements vary by the kind of test each laboratory conducts. Tests are categorized as waived, moderate complexity or high complexity. CLIA requires all laboratory testing sites to have one of the following certificates to legally perform clinical laboratory testing:

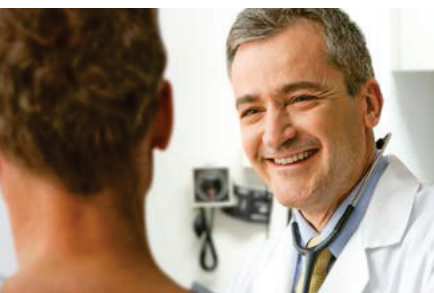
- Certificate of Waiver
- Certificate of Registration
- Certificate of Accreditation
- Certificate for Physician-Performed Microscopy
- Certificate of Compliance

Claims for laboratory services may be denied if the CLIA information is missing, invalid or not within the scope of the awarded CLIA Certificate per the CLIA ID number reported on the claim. Reporting of the modifier QW when billing for CLIA waived tests also may be required based on the level of CLIA certification the laboratory has obtained. Claims that are denied for missing information may be resubmitted with the required information.



For more information about the CLIA requirements and test complexity categories, please **visit cms.hhs.gov/clia**.





Front & Center

Dental Clinical Policy & Coverage Guideline Updates

On April 1, 2016, UnitedHealthcare began publishing Dental Clinical Policies and Coverage Guidelines to help administer dental plan benefits. These policies and guidelines apply to all UnitedHealthcare dental plans except certain custom groups. They serve as the clinical foundation for making dental coverage determinations and support our clinical dental programs.



A complete library of **Dental Clinical Policies and Coverage Guidelines** is available at **UnitedHealthCareOnline.com > Tools & Resources > Policies, Protocols and Guides > Dental Clinical Policies and Coverage Guidelines.**

The Dental Clinical Policies and Coverage Guidelines do not replace the current National Standardized Commercial Dental Claim Utilization Review Criteria located at UnitedHealthcare Dental Benefit Providers (DBP).com > Resources > **Clinical Guidelines**. As dental clinical policies and coverage guidelines are developed or modified, the National Standardized Commercial Dental Claim Utilization Review Criteria is updated in accordance with the dental policy or coverage guideline document.

UnitedHealthcare publishes monthly editions of the **Dental Policy Update Bulletin**, an online resource that provides notices of new and updated Dental Clinical Policies and Coverage Guidelines. A new edition of the bulletin is published on the first day of each month at UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Dental Clinical Policies & Coverage Guidelines > **Update Bulletin**. As a supplemental reminder to the detailed policy update summaries announced in the Dental Policy Update Bulletin, a list of recently approved, revised and/or retired Dental Policies and Coverage Guidelines is also provided in the Network Bulletin.



For complete details on the policy updates listed in the following table, please refer to the **July 2016 UnitedHealthcare Dental Policy Update Bulletin** at UnitedHealthCareOnline.com > Tools & Resources > Policies, Protocols and Guides > Dental Clinical Policies & Coverage Guidelines > Update Bulletin.

Policy Title	Policy Type
NEW (Effective Aug. 1, 2016)	
Core Buildup, Post and Core and Pin Retention	Coverage Guideline
Other Restorative Procedures	Coverage Guideline
Sealants	Coverage Guideline
Single Tooth Direct Restorations	Coverage Guideline

Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





UnitedHealthcare Commercial

PCORs—Use Regularly to Track Preventive Health Care Status

UnitedHealthcare supports your efforts to close open care opportunities so you can achieve positive health outcomes for your patients and meet quality care standards. To make reaching these goals easier, we're pleased to introduce the monthly Patient Care Opportunity Report (PCOR) for care providers participating in our Commercial network.

The personalized PCOR can be used to identify and track UnitedHealthcare members with open care opportunities. It gives you easy access to actionable patient data, reporting and resources at a group or health system, physician and member level. Information can be filtered to meet the specific needs of your practice.

If you currently receive a PCOR for your patients who are UnitedHealthcare Medicare Advantage members, you'll soon receive a letter on how to access the PCOR for Commercial members. Your PCOR PIN number will work for both the Medicare Advantage and Commercial versions of the PCOR. While the structure and format of the Commercial report is similar to the Medicare Advantage report, the measures and performance measurements differ in some cases.

If you haven't received a PCOR before, you'll receive a letter with the information and PIN number needed to access the report. The Commercial PCOR provides detailed instructions on how to interpret and act on the information in the report.



If you have any questions, please contact your Network Account Manager or Provider Advocate. Or, call our Provider Call Center at **877-842-3210**.





UnitedHealthcare Commercial

Medical Policy, Drug Policy and Coverage Determination Guideline



For complete details on the policy updates listed in the following table, please refer to the **July 2016 Medical Policy Update Bulletin** at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Medical & Drug Policies and Coverage Determination Guidelines > Medical Policy Update Bulletin](#).

Policy Title	Policy Type	Effective Date
UPDATED/REVISED		
Apheresis	Medical	July 1, 2016
Benlysta® (Belimumab)	Drug	July 1, 2016
Breast Reconstruction Post Mastectomy	CDG	Sept. 1, 2016
Breast Reduction Surgery	CDG	Aug. 1, 2016
Bronchial Thermoplasty	Medical	July 1, 2016
Continuous Glucose Monitoring and Insulin Delivery for Managing Diabetes	Medical	Aug. 1, 2016
Cosmetic and Reconstructive Procedures	CDG	Sept. 1, 2016
Embolization of the Ovarian and Iliac Veins for Pelvic Congestion Syndrome	Medical	Aug. 1, 2016
Fecal DNA Testing	Medical	July 1, 2016
Genetic Testing for Hereditary Breast and/or Ovarian Cancer Syndrome (HBOC)	Medical	Aug. 1, 2016
Home Traction Therapy	Medical	July 1, 2016
Light and Laser Therapy for Cutaneous Lesions and Pilonidal Disease	Medical	Aug. 1, 2016
Mechanical Stretching and Continuous Passive Motion Devices	Medical	Aug. 1, 2016
Meniscus Implant and Allograft	Medical	July 1, 2016
Mifeprex® (Mifepristone, RU-486)	Drug	Aug. 1, 2016
Molecular Profiling to Guide Cancer Treatment	Medical	July 1, 2016
Motorized Spinal Traction	Medical	July 1, 2016
Obstructive Sleep Apnea Treatment	Medical	Oct. 1, 2016

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UnitedHealthcare Commercial

Medical Policy, Drug Policy and Coverage Determination Guideline

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Policy Title	Policy Type	Effective Date
Oncology Medication Clinical Coverage	Drug	July 1, 2016
Pectus Deformity Repair	CDG	Sept. 1, 2016
Preterm Labor Management	Medical	Aug. 1, 2016
Preventive Care Services	CDG	Oct. 1, 2016
Private Duty Nursing Services (PDN)	CDG	Aug. 1, 2016
Repository Corticotropin Injection (H.P. Acthar Gel®)	Medical	July 1, 2016
Rhinoplasty and Other Nasal Surgeries	CDG	Sept. 1, 2016
Rituxan® (Rituximab)	Drug	July 1, 2016
Soliris® (Eculizumab)	Drug	July 1, 2016
Total Knee Replacement Surgery (Arthroplasty)	Medical	Sept. 1, 2016
Transcatheter Heart Valve Procedures	Medical	Sept. 1, 2016
Umbilical Cord Blood Harvesting and Storage for Future Use	Medical	July 1, 2016
Vaccines	Drug	July 1, 2016

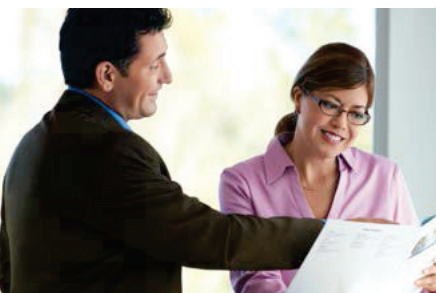
Note: The inclusion of a service or procedure on this list does not imply UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





UnitedHealthcare Commercial Reimbursement Policies

Unless otherwise noted, the following reimbursement policies apply to services reported using the 1500 Health Insurance Claim Form (CMS-1500) or its electronic equivalent or its successor form. UnitedHealthcare reimbursement policies do not address all factors that affect reimbursement for services rendered to UnitedHealthcare members, including legislative mandates, member benefit coverage documents, UnitedHealthcare medical or drug policies, and the UnitedHealthcare Physician, Health Care Professional, Facility and Ancillary Provider Administrative Guide. Meeting the terms of a particular reimbursement policy is not a guarantee of payment. Once implemented, the policies may be viewed in their entirety at **[UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Reimbursement Policies - Commercial](#)**. In the event of an inconsistency between the information provided in the Network Bulletin and the posted policy, the posted policy prevails.



UnitedHealthcare Commercial Reimbursement Policies

Revision to the Multiple Procedure Policy for Procedure Codes Assigned a Relative Value of 0.00

To further align with the Centers for Medicare and Medicaid Services (CMS), UnitedHealthcare will begin to consider all procedure codes with a CMS Multiple Procedure Indicator (MPI) of 2 or 3 as indicated in the National Physician Fee Schedule (NPPFS) Relative Value File, as eligible for multiple procedure reductions when reported with other procedure codes subject to reductions. Medical and surgical services include pre-procedure and post-procedure work, as well as generic services integral to the standard medical/surgical service. When multiple procedures are performed on the same day, by the same group physician and/or other health care professional, a reduction in reimbursement for secondary and subsequent procedures will occur as full payment would represent reimbursement of duplicative components of the primary procedure.

Currently, procedure codes with an MPI of 2 or 3 that do not have a CMS RVU assigned and are not assigned a gap value are excluded from ranking by UnitedHealthcare. Effective in the fourth quarter of 2016, UnitedHealthcare will begin considering procedure codes with a CMS MPI of 2 or 3 and with an RVU of 0.00 as secondary or subsequent procedures and therefore, reducible based on the CMS multiple procedure concept.

As a reminder, procedures assigned a CMS MPI of 2 or 3 are subject to the multiple procedure concept and:

- The procedure code assigned the highest RVU will be considered the primary procedure.
- If two or more procedure codes are assigned an RVU of 0.00, the procedure code with the larger billed charge will be considered the higher ranked procedure.

With the exception of those services appropriately reported with modifier 78, no other modifiers are given consideration in determining whether the multiple procedure reduction is applied.



The revised Multiple Procedure Policy reflecting this change will be accessible in mid-November 2016 at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides](#).

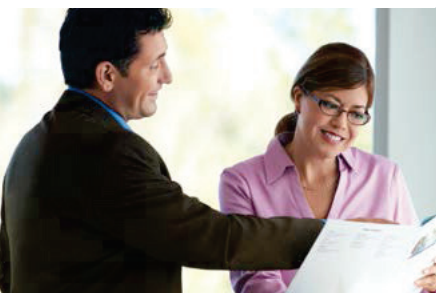
Revised Reimbursement Policy: Pediatric and Neonatal Critical and Intensive Care Services

Effective as of July 3, 2016 process dates, UnitedHealthcare revised the Pediatric and Neonatal Critical and Intensive Care Services Policy for UnitedHealthcare Commercial plans. The revision allows separate reimbursement of certain Evaluation and Management (E/M) codes for services provided in addition to pediatric and neonatal critical care by physicians and/or other health care professionals in the same group practice on the same date of service for the same patient.

While the American Medical Association (AMA) Current Procedural Terminology (CPT®) code book gives specific direction for the appropriate reporting of pediatric and neonatal critical and intensive care services and time-based critical care codes, the revised policy allows separate reimbursement for E/M codes 99221-99233, 99468-99480 and 99291 when provided in addition to pediatric and neonatal critical care (codes 99468-99476 and 99477-99480) by a second individual of a different specialty in the same group practice on the same date of service for the same patient.

Separate reimbursement for these E/M services may be subject to editing in other reimbursement policies such as the Same Day Same Service and Rebundling policies. These policies are available for review at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Reimbursement Policies - Commercial](#).





UnitedHealthcare Commercial Reimbursement Policies

Revision to Ambulance Policy

The Ambulance Policy for UnitedHealthcare Commercial plans will be revised in the fourth quarter of 2016. The revised policy will consider reimbursement for an Advanced Life Support, Level 2 transport (HCPCS A0433), when certain criteria are met.

According to the Centers for Medicare & Medicaid Services (CMS), Advanced Life Support, Level 2 (ALS2) is transportation by ground ambulance and the provision of medically necessary supplies and services, including:

- At least three separate administrations of one or more medications by intravenous push/bolus or by continuous infusion (excluding crystalloid fluids); **or**
- Ground ambulance transport, medically necessary supplies and services, and the provision of at least one of these ALS2 procedures:
 - Manual defibrillation/cardioversion
 - Endotracheal intubation
 - Central venous line
 - Cardiac pacing
 - Chest decompression
 - Surgical airway
 - Intraosseous line

Based on an interpretation of these CMS guidelines, the following list of CPT codes were developed to identify the services outlined as Criteria 1 or Criteria 2:

Criteria 1 - Three separate administrations by intravenous push/bolus or continuous infusion:

Description of Service	Code
Intravenous push	96374, +96375, +96376
Continuous infusion	96365, +96366, +96367, +96368

Criteria 2 - The provision of at least one of these ALS2 procedures:

Description	Code
Manual defibrillation/cardioversion	92960, 92961
Endotracheal intubation	31500
Central venous line	36555, 36556, 36568, 36569
Cardiac pacing	92953
Chest decompression	92950
Surgical airway	31603, 31605
Intraosseous line	36680, 36000

These services are **not** considered included as part of an ambulance transportation service and can be allowed separately.

UnitedHealthcare will only consider reimbursement of an ALS2 transport (HCPCS A0433) when it is reported with services outlined in either Criteria 1 or Criteria 2 as defined by CMS.

Ambulance transport services that do not include the services described in criteria 1 or 2 above should be reported with a more appropriate ambulance transport code.

The revised Ambulance Policy reflecting this change will be accessible in mid-November 2016 at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides.](#)





UnitedHealthcare Community Plan Revision to the Incontinence Supplies Policy

UnitedHealthcare Community Plan currently denies incontinence supplies when one or more diagnosis codes listed in the Incontinence Supply Policy are the **ONLY** diagnosis codes on the claim. Claims for incontinence supplies must contain a diagnosis reflecting the medical condition causing incontinence.

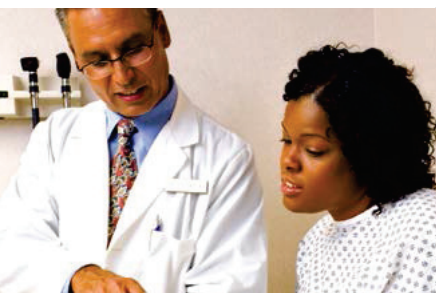
For claims processed on or after Aug. 20, 2016 with a date of service on or after Oct. 1, 2015, there must be more than one ICD-10 diagnosis code billed for incontinence supplies. An ICD-10 diagnosis code from the approved ICD-10 diagnosis codes list in the policy and an ICD-10 diagnosis code reflecting the condition causing incontinence both must be present on the claim. If only incontinence diagnosis codes are on the claim, all incontinence supplies will be denied.

Claims with dates of service prior to Oct. 1, 2015 will not be affected by this change and will continue to be denied if one or more of the codes from the approved ICD-9 diagnosis list are the **ONLY** diagnosis codes on the claim.

In addition, UnitedHealthcare Community Plan will be updating the ICD-10 Diagnosis Codes List with additional codes reflecting the type of incontinence. For the current list of diagnosis codes, please refer to the Incontinence Supply Policy posted under the Reimbursement Policy section at **UHCCommunityPlan.com > For Health Care Professionals > Select Your State > Reimbursement Policy**.

As with all UnitedHealthcare Community Plan policies, other factors affecting reimbursement may supplement, modify or in some cases supersede this policy. These factors include, but are not limited to, federal and/or state regulatory requirements, physician or other provider contracts and/or the member's benefit coverage documents.





UnitedHealthcare Community Plan

Outpatient Injectable Chemotherapy Prior Authorization Program for UnitedHealthcare Community Plan in Ohio, Michigan, Mississippi and Wisconsin

Effective **Oct. 1, 2016**, UnitedHealthcare Community Plan members in Ohio, Mississippi, Michigan and Wisconsin will require prior authorization for injectable outpatient chemotherapy drugs given for a cancer diagnosis.

This requirement will NOT apply to UnitedHealthcare Community Plan members in Kansas, as previously communicated on July 1, 2016.

This requirement has been in place for UnitedHealthcare's Commercial members since June 1, 2015.

Prior authorization will be required for:

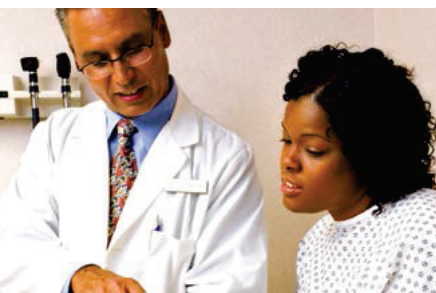
- Chemotherapy injectable drugs (J9000 - J9999), Leucovorin (J0640) and Levoleucovorin (J0641)
- Chemotherapy injectable drugs that have a Q code
- Chemotherapy injectable drugs that have not yet received an assigned code and will be billed under a miscellaneous Healthcare Common Procedure Coding System (HCPCS) code
- All outpatient injectable chemotherapy drugs started after the chemotherapy prior authorization effective date
- Adding a new injectable chemotherapy drug to a regimen

If a UnitedHealthcare Community Plan member in Ohio, Mississippi, Michigan or Wisconsin received injectable chemotherapy drugs in an outpatient setting July 1 through Sept. 30, 2016, you do not need to submit a prior authorization request until a new chemotherapy drug will be administered. We will authorize the chemotherapy regimen the member was receiving prior to Oct. 1, 2016 and the authorization will be effective until Sept. 30, 2017.



To request prior authorization, go to **[UnitedHealthcareOnline.com](https://www.unitedhealthcareonline.com)** > **Notifications/Prior Authorizations > Oncology Authorizations Submission & Status** or **call 866-889-8054**.





UnitedHealthcare Community Plan

Preferred Drug List Changes

UnitedHealthcare Community Plan routinely evaluates prescription benefit coverage to offer members clinically- and cost-effective medications. As a result, the UnitedHealthcare Community Plan Preferred Drug List (PDL) is updated each quarter.

Effective July 1, 2016, we implemented changes to medications used to treat Hepatitis C that will affect our Medicaid/Children's Health Insurance Program (CHIP) members in Hawaii, Maryland, New Jersey, New Mexico, New York, Ohio and Pennsylvania. For Medicaid/CHIP members in Louisiana, the changes became **effective July 15, 2016**.

Hepatitis C medication changes:

- **Harvoni (ledipasvir/sofosbuvir) tablet** was removed from UnitedHealthcare Community Plan's PDL.

Members treated with Harvoni prior to July 1, 2016 were not affected by this change and were allowed to continue their existing regimen. However, any new Harvoni users as of July 1, 2016 (July 15 in Louisiana) are required to use another alternative on the Preferred Drug List.

All Hepatitis C drugs will continue to require prior authorization.

- **Zepatier (elbasvir/grazeoprevir)** was added to UnitedHealthcare Community Plan's PDL as a preferred alternative to treat Hepatitis C.

PDL Table: (Hepatitis C medications)

Previous PDL	New PDL
▪ Harvoni – preferred for Genotypes** 1 and 4 ▪ Sovaldi – preferred for Genotypes 2 and 3 ▪ Daklinza – preferred for Genotype 3 ▪ Ribavirin 200 mg caps and tabs ▪ Pegasys	▪ Zepatier – preferred for Genotypes 1 and 4 ▪ Sovaldi – preferred for Genotypes 2 and 3 ▪ Daklinza – preferred for Genotype 3 ▪ Ribavirin 200 mg caps and tabs ▪ Pegasys

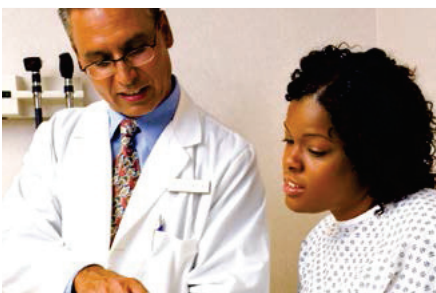
** Genotype means the variant of Hepatitis C virus



For state-specific details, please visit UHCCommunityPlan.com > **For Health Care Professionals > Select Your State > Pharmacy Program.**

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UnitedHealthcare Community Plan

Preferred Drug List Changes

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Effective Aug. 1, 2016, the following changes have been made to UnitedHealthcare Community Plan's PDL in some states.

New medications added to the PDL:

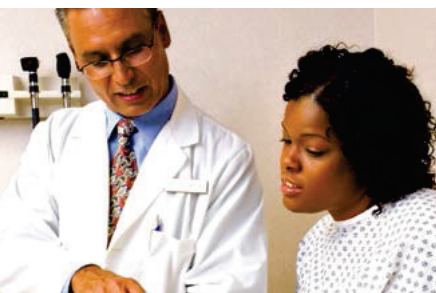
Strensiq (asfotase alfa) injection	Prior authorization required
Cotellic (cobimetinib) tablets	Prior authorization required
Genvoya (elvitegravir/cobicistat/emtricitabine/tenofovir alafenamide fumarate) tablets	Prior authorization required*
Ninlaro (ixazomib) capsules	Prior authorization required
Alecensa (alectinib) capsules	Prior authorization required
Adempas (riociguat) tablets	Prior authorization required
Opsumit (macitentan) tablets	Prior authorization required
Gleostine (lomustine) 5mg capsules	
Narcan (naloxone) nasal spray	

*Prior authorization not required for Medicaid/CHIP members in Louisiana.

Modifications to the PDL:

Capecitabine (Xeloda) tablet	Medical necessity review not required. Prior authorization required.
Lansoprazole (Prevacid) tablet	Remove Step Therapy, will be open-access drug
Levocetirizine (Xyzal) tablet	Remove Step Therapy, will be open-access drug





UnitedHealthcare Community Plan

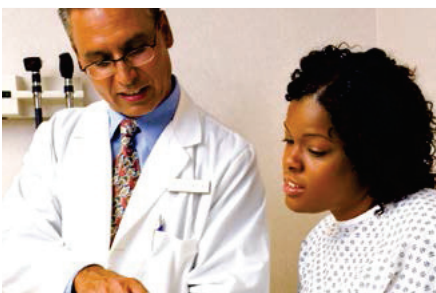
Medical Policy & Coverage Determination Guideline Updates



For complete details on the policy updates listed in the following table, please refer to the **July 2016 Medical Policy Update Bulletin** at UHCCommunityPlan.com > **For Health Care Professionals** > **Select Your State** > **Provider Information** > **UnitedHealthcare Community Plan Medical Policies and Coverage Determination Guidelines**.

Policy Title	Policy Type	Effective Date
UPDATED/REVISED		
Apheresis	Medical	July 1, 2016
Breast Reconstruction Post Mastectomy	CDG	Sept. 1, 2016
Breast Reduction Surgery	CDG	Aug. 1, 2016
Bronchial Thermoplasty	Medical	July 1, 2016
Continuous Glucose Monitoring and Insulin Delivery for Managing Diabetes	Medical	Aug. 1, 2016
Cosmetic and Reconstructive Procedures	CDG	Sept. 1, 2016
Embolization of the Ovarian and Iliac Veins for Pelvic Congestion Syndrome	Medical	Sept. 1, 2016
Emergency Health Services and Urgent Care Center Services (Maryland Only)	CDG	Sept. 1, 2016
Extracorporeal Shock Wave Therapy (ESWT)	Medical	Aug. 1, 2016
Fecal DNA Testing	Medical	July 1, 2016
Gastrointestinal Motility Disorders, Diagnosis and Treatment	Medical	Aug. 1, 2016
Genetic Testing for Hereditary Breast and/or Ovarian Cancer Syndrome (HBOC)	Medical	Aug. 1, 2016
Home Traction Therapy	Medical	July 1, 2016

[Continued >](#)

UnitedHealthcare Community Plan

Medical Policy & Coverage Determination Guideline Updates

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Policy Title	Policy Type	Effective Date
Light and Laser Therapy for Cutaneous Lesions and Pilonidal Disease	Medical	Sept. 1, 2016
Mechanical Stretching and Continuous Passive Motion Devices	Medical	Aug. 1, 2016
Meniscus Implant and Allograft	Medical	July 1, 2016
Motorized Spinal Traction	Medical	July 1, 2016
Obstructive Sleep Apnea Treatment	Medical	Oct. 1, 2016
Omnibus Codes	Medical	Aug. 1, 2016
Oral and Enteral Nutrition	CDG	Aug. 1, 2016
Pectus Deformity Repair	CDG	Sept. 1, 2016
Preterm Labor Management	Medical	Aug. 1, 2016
Private Duty Nursing Services (PDN)	CDG	Sept. 1, 2016
Rhinoplasty and Other Nasal Surgeries	CDG	Sept. 1, 2016
Sodium Hyaluronate	Medical	Aug. 1, 2016
Surgical and Ablative Procedures for Venous Insufficiency and Varicose Veins	Medical	Aug. 1, 2016
Total Knee Replacement Surgery (Arthroplasty)	Medical	Sept. 1, 2016
Transcatheter Heart Valve Procedures	Medical	Sept. 1, 2016
Umbilical Cord Blood Harvesting and Storage for Future Use	Medical	July 1, 2016

Note: The inclusion of a service or procedure on this list does not imply that UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





UnitedHealthcare Medicare Solutions

Skilled Nursing Facilities Requirement for Outpatient Services

Effective Sept. 1, 2016, UnitedHealthcare Medicare Advantage plans will begin requiring skilled nursing facilities to include HCPCS codes on claims for outpatient services – the only exception will be claims for physical, occupational and speech therapy. Claims submitted on or after this date without HCPCS codes will be denied and must be resubmitted with correct coding. This change will enable UnitedHealthcare to consistently apply appropriate benefits to claims and help ensure accurate member cost-sharing. This change aligns with our current policy on complete coding requirements, which can be found at **[UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > Administrative Guides](#)**.





UnitedHealthcare Medicare Solutions

Medicare Advantage Coverage Summary Updates



For complete details on the policy updates listed in the following table, please refer to the **July 2016 Medicare Advantage Coverage Summary Update Bulletin** at [UnitedHealthcareOnline.com > Tools & Resources > Policies, Protocols and Guides > UnitedHealthcare Medicare Advantage Coverage Summaries > Update Bulletin](#).

Policy Title
UPDATED/REVISED (Approved on June 21, 2016)
Age Related Macular Degeneration (AMD) Therapy (Macugen®, Lucentis®, Avastin®, EYLEA®)
Biofeedback
Carotid Procedures and Testing
Durable Medical Equipment (DME), Prosthetics, Corrective Appliances/Orthotics (Non-Foot Orthotics) and Medical Supplies Grid
Durable Medical Equipment, Prosthetics, Corrective Appliances/Orthotics and Medical Supplies
Gastroesophageal and Gastrointestinal (GI) Services and Procedures
Glaucoma Surgical Treatments
Incontinence: Urinary and Fecal Incontinence, Diagnosis and Treatments
Joints and Joint Procedures
Medications/Drugs (Outpatient/Part B)
Pain Management and Pain Rehabilitation
Rehabilitation: Cardiac Rehabilitation Services (Outpatient)
Second and Third Opinions
Transcatheter Heart Valve Procedures
Veteran Administration (VA) and Indian Health Services (IHS)
REPLACED (Approved on June 21, 2016)
Arthroscopic Lavage and Debridement Treatment of the Knee(s)

Note: The inclusion of a service or procedure to this list does not imply that UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





Doing Business Better

Coordination of Care through Controlled Substance Monitoring

UnitedHealthcare's Controlled Substances Monitoring Program promotes patient well-being through coordination of care by identifying members who may benefit from having their narcotic pain management regimens reviewed and evaluated by their care providers. Through the program, care providers are informed about specific members who are receiving opioids and other medications with a high potential for misuse or abuse.

The program uses the following criteria to help identify potential substance abuse through pharmacy claims data analysis:

- Overlapping use of multiple controlled substances
- Multiple providers prescribing the same or similar controlled substances
- Multiple pharmacies dispensing the same or similar controlled substances

Care providers whose patients are identified through this program receive an introductory letter and a comprehensive member-specific report that includes the clinical issue of concern, prescription utilization details for the medications involved and recommended action. These care providers are encouraged to contact the members to discuss and re-evaluate their narcotic pain management regimens and to coordinate appropriate treatment, if indicated.

Care providers also can request access to state reporting systems through the Alliance of States with Prescription Monitoring Programs. The systems allow providers to identify other patients who may be paying cash for opioids or controlled substances in an effort to avoid identification.





UnitedHealthcare Affiliates

SignatureValue/UnitedHealthcare Benefits Plan of California Benefit Interpretation Policy Updates



For complete details on the policy updates listed in the following table, please refer to the **July 2016 SignatureValue™ / UnitedHealthcare Benefits Plan of California Benefit Interpretation Policy Update Bulletin** at **UHCWest.com > Provider Log In > Library > Resource Center > Guidelines & Interpretation Manuals**.

Policy Title
UPDATED/REVISED (Effective Aug. 1, 2016)
Educational Programs for Members
Hospice
Preventive Care Services
Services/Complications Related to Non-Covered Services
Treatment of Temporomandibular Joint (TMJ) Disorders
Veteran's Administration (VA)
Wheelchairs and Accessories
REPLACED (Effective Aug. 1, 2016)
Respite Care

Note: The inclusion of a service or procedure to this list does not imply that UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





UnitedHealthcare Affiliates

SignatureValue/UnitedHealthcare Benefits Plan of California Medical Management Guideline Updates



For complete details on the policy updates listed in the following table, please refer to the **July 2016 SignatureValue/UnitedHealthcare Benefits Plan of California Medical Management Guidelines Update Bulletin** at **UHCWest.com > Provider Log In > Library > Resource Center > Guidelines & Interpretation Manuals**.

Title	Effective Date
UPDATED/REVISED	
Apheresis	July 1, 2016
Breast Reduction Surgery	Sept. 1, 2016
Bronchial Thermoplasty	July 1, 2016
Clinical Trials	July 1, 2016
Continuous Glucose Monitoring and Insulin Delivery for Managing Diabetes	Aug. 1, 2016
Embolization of the Ovarian and Iliac Veins for Pelvic Congestion Syndrome	Aug. 1, 2016
Fecal DNA Testing	July 1, 2016
Genetic Testing for Hereditary Breast and/or Ovarian Cancer Syndrome (HBOC)	Aug. 1, 2016
Gynecomastia Treatment	Aug. 1, 2016
Home Traction Therapy	July 1, 2016
Light and Laser Therapy for Cutaneous Lesions and Pilonidal Disease	Aug. 1, 2016
Mechanical Stretching and Continuous Passive Motion Devices	Aug. 1, 2016
Meniscus Implant and Allograft	July 1, 2016
Molecular Profiling to Guide Cancer Treatment	July 1, 2016
Motorized Spinal Traction	July 1, 2016

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UnitedHealthcare Affiliates

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Policy Title	Policy Type
Obstructive Sleep Apnea Treatment	Oct. 1, 2016
Preterm Labor Management	Aug. 1, 2016
Total Knee Replacement Surgery (Arthroplasty)	Sept. 1, 2016
Transcatheter Heart Valve Procedures	Sept. 1, 2016
Umbilical Cord Blood Harvesting and Storage for Future Use	July 1, 2016

Note: The inclusion of a service or procedure to this list does not imply that UnitedHealthcare provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.





UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates



For complete details on the policy updates listed in the following table, please refer to the **July 2016 Policy Update Bulletin** at **OxfordHealth.com > Providers > Tools & Resources > Medical Information > Medical and Administrative Policies > Policy Update Bulletin**.

Policy Title	Policy Type	Effective Date
UPDATED/REVISED		
Actemra® (Tocilizumab) Injection for Intravenous Infusion	Clinical	July 1, 2016
Ambulance	Reimbursement	July 18, 2016
Athletic Pubalgia Surgery	Clinical	Aug. 1, 2016
Bilateral Procedures	Reimbursement	Aug. 8, 2016
Breast Imaging for Screening and Diagnosing Cancer	Clinical	Aug. 1, 2016
Breast Reduction Surgery	Clinical	Aug. 1, 2016
Clinical Review Policy	Administrative	Aug. 1, 2016
Clinical Trials	Clinical	July 1, 2016
Continuous Glucose Monitoring and Insulin Delivery for Managing Diabetes	Clinical	Aug. 1, 2016
Corneal Hysteresis and Intraocular Pressure Measurement	Clinical	July 1, 2016
Disclosure Policy	Administrative	July 1, 2016
Discontinued Procedures	Reimbursement	July 1, 2016
Drug Coverage Criteria - New and Therapeutic Equivalent Medications	Clinical	Aug. 1, 2016

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UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates

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Policy Title	Policy Type	Effective Date
Drug Coverage Guidelines	Clinical	June 27, 2016 July 1, 2016 Aug. 1, 2016
Embolization of the Ovarian and Iliac Veins for Pelvic Congestion Syndrome	Clinical	Aug. 1, 2016
Extracorporeal Shock Wave Therapy (ESWT)	Clinical	Aug. 1, 2016
Fecal DNA Testing	Clinical	July 1, 2016
From – To Policy	Reimbursement	July 3, 2016
Gastrointestinal Motility Disorders, Diagnosis and Treatment	Clinical	Aug. 1, 2016
Genetic Testing for Hereditary Breast and/or Ovarian Cancer Syndrome (HBOC)	Clinical	Aug. 1, 2016
Global Days	Reimbursement	July 18, 2016
Gynecomastia Treatment	Clinical	July 1, 2016
Immune Globulin (IVIG and SCIG)	Clinical	Aug. 1, 2016
Injectable Chemotherapy Drugs: Application of NCCN Clinical Practice Guidelines	Clinical	Aug. 1, 2016
Intraoperative Hyperthermic Intraperitoneal Chemotherapy (HIPEC)	Clinical	July 1, 2016
Macular Degeneration Treatment Procedures	Clinical	July 1, 2016
Magnetoencephalography and Magnetic Source Imaging for Specific Neurological Applications	Clinical	July 1, 2016
Mechanical Stretching and Continuous Passive Motion Devices	Clinical	Aug. 1, 2016
Mifeprex® (Mifepristone, RU-486)	Clinical	Aug. 1, 2016
Obstructive Sleep Apnea Treatment	Clinical	Oct. 1, 2016
Orencia® (Abatacept) - Injection for Intravenous Infusion	Clinical	July 1, 2016
Precertification Exemptions for Outpatient Services	Administrative	Aug. 1, 2016
Preterm Labor Management	Clinical	Aug. 1, 2016
Preventive Care Services	Clinical	July 1, 2016

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UnitedHealthcare Affiliates

Oxford® Medical and Administrative Policy Updates

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Policy Title	Effective Date	
Repository Corticotropin Injection (H.P. Acthar Gel®)	Clinical	Aug. 1, 2016
Rituxan® (Rituximab)	Clinical	Aug. 1, 2016
Same Day/Same Service	Reimbursement	July 1, 2016
Site of Service Guidelines for Certain Outpatient Surgical Procedures	Clinical	Oct. 1, 2016
Sodium Hyaluronate	Clinical	Aug. 1, 2016
Soliris® (Eculizumab)	Clinical	July 1, 2016
Specialty Medication Administration - Site of Care Review Guidelines	Clinical	July 1, 2016
Split Surgical Package	Reimbursement	July 1, 2016
Stelara® (Ustekinumab) Injection for Intravenous Infusion	Clinical	July 1, 2016
Surgical and Ablative Procedures for Venous Insufficiency and Varicose Veins	Clinical	Aug. 1, 2016
Transpupillary Thermotherapy	Clinical	July 1, 2016
Vaccines	Clinical	Aug. 1, 2016
RETIRED		
Prescription Drug Quantity Duration (QD) and Quantity Level Limitations (QLL)	Clinical	Aug. 1, 2016

Note: The inclusion of a service or procedure to this list does not imply that Oxford provides coverage for the service or procedure. In the event of an inconsistency or conflict between the information in this bulletin and the posted policy, the provisions of the posted policy prevail.

Oxford HMO products are underwritten by Oxford Health Plans (NY), Inc., Oxford Health Plans (NJ), Inc. and Oxford Health Plans (CT), Inc. Oxford insurance products are underwritten by Oxford Health Insurance, Inc.

